

HOW TO OPEN AN ASSISTED LIVING HOME FOR ADULTS WITH BRAIN INJURY in Arizona

An ADHS-licensed Assisted Living Home providing Directed Care — funded through ALTCS.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact & fee should be re-verified before you rely on it.

The Arizona model — read this first

In Arizona, a home that cares for adults with brain injury is licensed by the Arizona Department of Health Services (ADHS) as an Assisted Living Home (ten or fewer residents) or Assisted Living Center (eleven or more) under Arizona Administrative Code Title 9, Chapter 10, Article 8. It is authorized to provide Directed Care — the level for residents who cannot direct their own care — and it is paid through the Arizona Long Term Care System (ALTCS) by contracting with the ALTCS health plans. There is one path: get the home licensed, certify a manager, and contract as a provider yourself. This guide walks that whole path, broken down so a first-timer can follow it.

Accreditation — strongly recommended, and how we build. Arizona does not require national accreditation to hold an assisted living license, but for a brain-injury program it is a real advantage: CARF (which offers a Brain Injury specialty) and The Joint Commission signal quality to ALTCS health plans, hospital discharge planners, and families, and can strengthen your contracts and rates. Every PolicyReady product is built to those standards behind the scenes. Treat accreditation as a goal from day one (see Parts 7 & 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened a facility — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with ADHS, the NCIA Board, and AHCCCS/ALTCS (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, contracted, paid home. Each part below is one rung.

1	Decide this is your business	A licensed, 24-hour directed-care home — durable, census-based revenue (Part 1)
2	Form the business & insure	Entity, EIN, bank, and the right insurance — broken all the way down (Part 3)
3	Secure & prepare the home	Zoning, and the Article 8 physical-plant specs (Parts 3–4)
4	Certify your manager	The NCIA Board assisted living facility manager certificate (Part 5)
5	Build & train caregivers	62-hour caregiver training, first aid/CPR, fingerprint clearance (Part 5)
6	Write your policies	P&P Manual covering the 23 required-policy areas of R9-10-803(C) (Parts 4–6)
7	Get the ADHS license	Apply in the LMS portal; pass the inspection; perpetual license (Part 6)
8	Enroll & contract for ALTCS	AHCCCS APEP + contract the ALTCS health plans (Part 8)
9	Get your first residents	ALTCS case managers +

		hospital/Barrow discharge planners (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. *Certify your manager early — you cannot operate or get licensed without a certified manager, and the training + exam take time. Do the cheap, reversible steps (form the entity, lease) first, and get fingerprint clearance cards started for everyone right away.*

Before you begin — a realistic picture

Be honest with yourself about the scale of this before you spend money. You are opening a licensed, 24-hour healthcare setting for adults who cannot direct their own care — not a rental property. Expect roughly four months from a finished application to a license, and more time before that to certify a manager, ready the home to Article 8 specs, and train caregivers; then more time still to enroll with AHCCCS and sign your first ALTCS plan contract before a single dollar comes in. You will front real costs — the home, insurance, the manager's training and exam, caregiver training, fingerprint cards, and ADHS fees — while earning nothing, so plan working capital to cover several months of rent, payroll, and utilities on top of startup. The reward is durable: a small directed-care home fills a genuine gap in Arizona, brain-injury residents tend to stay for years, and ALTCS is a stable, census-based payer. The people who succeed treat this as a business and a clinical operation at once — patient, organized, and willing to follow the rules exactly.

Here is the roadmap. Parts 1–2 explain what this business is and who regulates you. Part 3 walks you through forming the entity, banking, insurance, zoning, and your billing IDs, step by step. Part 4 gives the actual Article 8 numbers for the building — bedrooms, bathrooms, temperature, fire safety — and what each means for you. Part 5 covers every staff role, the manager certification (start it first), the 62-hour caregiver training, and fingerprint clearance. Part 6 takes you through the ADHS license application and inspection. Part 7 follows a resident from referral to discharge. Part 8 is how you actually get paid through ALTCS. Part 9 lays out costs and the timeline, Part 10 shows how to earn your first referrals, and Part 11 collects every phone number and verify-this question in one place. Read it once end to end, then work the parts in order.

PART 1

What This Business Is

In short: An ADHS-licensed assisted living HOME (≤ 10 residents) providing Directed Care — the level built for brain-injury adults who can't direct their own care. It's a real-estate + 24-hour-care business.

Arizona draws the line by size: an assisted living HOME serves ten or fewer residents; an assisted living CENTER serves eleven or more. Both are licensed to provide one or more levels of care tied to how much help residents need.

The three levels of care (A.R.S. § 36-401)

Level	Who it's for
Supervisory care	General supervision — daily awareness, ability to intervene in a crisis, help with self-administered meds
Personal care	Hands-on help with daily living (bathing, dressing, eating, mobility) plus intermittent nursing/medication
Directed care	For residents incapable of recognizing danger, summoning help, expressing need, or making basic care decisions

Directed Care — the level that serves brain injury (R9-10-815)

Adults with brain injury who cannot direct their own care are served under Directed Care. It includes all personal care plus cognitive-stimulation programming to maximize functioning, personal-safety strategies, and measures to keep a resident who may wander safe. A resident's designated representative is used where the resident can't direct self-care, and no non-prescription medication may be given to a directed-care resident without a medical practitioner's order.

The brain-injury / psychotropic rule (R9-10-808(F)). A resident with a primary diagnosis of traumatic brain injury who receives directed care and is prescribed a psychotropic medication is NOT — on that basis alone — treated as needing behavioral health services. So a brain-injury resident is properly served in a directed-care assisted living home, without triggering the heavier behavioral-health regime.

“Memory care” is not a separate license. Arizona doesn't license a standalone memory-care category. A secured/restricted-egress home operates under the Directed Care authorization and must meet the egress-control requirements of R9-10-815(F) — a means of exit that either lets the resident reach a point ≥ 30 ft away while controlling or alerting staff, or a Special Egress-Control Device meeting the International Building Code. Plan the physical layout and the service plan around that rule.

Key terms. ADHS = AZ Dept. of Health Services (your licenser). ALTCS = Arizona Long Term Care System (Medicaid long-term care). AHCCCS = Arizona's Medicaid agency. Program Contractor = an ALTCS health plan. NCIA Board = the board that certifies your manager. FCC = Fingerprint Clearance Card. HNT = HCBS Needs Tool (the assessment). PCSP = Person-Centered Service Plan. NPI = National Provider Identifier. CARF/TJC = the two rehabilitation accreditors.

PART 2

Who Regulates You (and What Each One Does)

In short: ADHS licenses you, AHCCCS/ALTCS + the health plans pay you, the NCIA Board certifies your manager, and the Long-Term Care Ombudsman advocates for residents.

ADHS — Arizona Department of Health Services (Residential Facilities Licensing)

Your licenser. ADHS issues your assisted living license, writes the Article 8 standards, runs the online Licensing Management System (LMS) where you apply, and conducts the pre-licensure inspection and ongoing surveys. Phone 602-542-1025; azdhs.gov/licensing/residential-facilities.

AHCCCS / ALTCS and the health plans

AHCCCS is Arizona's Medicaid agency; ALTCS is its long-term-care program, run through managed care. You enroll as an AHCCCS provider (through the APEP portal) and then contract with the ALTCS health plans ("Program Contractors") that actually pay you. Each ALTCS member has a case manager who assesses needs and authorizes the directed-care service.

NCIA Board — the manager-certification board

The Arizona State Board of Examiners for Nursing Care Institution Administrators and Assisted Living Facility Managers ("NCIA Board") issues the assisted living facility manager certificate your manager must hold (Part 5). Phone 602-364-2374; nciaboard.az.gov.

Long-Term Care Ombudsman & Adult Protective Services

The Arizona Long-Term Care Ombudsman (through DES) investigates residents'-rights complaints — post their number for residents (602-542-6454, ext. 9). DES Adult Protective Services (APS) runs the APS Registry you must check on staff (since Jan 1, 2025) and takes abuse/neglect/exploitation reports.

PART 3

Form Your Business & Secure the Home — Step by Step

In short: *This is where people get stuck. We break each piece down: what it is, where to go, and exactly what to do.*

3.1 Form your legal entity

An “entity” is the legal business (usually an LLC) that holds the license and bank account and shields your personal assets. In Arizona you form it at the Arizona Corporation Commission (ACC), not the Secretary of State.

1. **Go to the ACC.** File online at ecorp.azcc.gov (the Corporations Division). azcc.gov · 602-542-3026.
2. **File Articles of Organization (LLC).** The online LLC filing fee is about \$85. Name the LLC and list a statutory agent (receives legal mail — can be you).
3. **Handle the publication step.** New AZ LLCs must publish a Notice of Formation in an approved newspaper — EXCEPT in Maricopa and Pima counties, where the ACC posts it for you. Check whether your county requires publication.
4. **Save your documents** and your entity number — the bank and ADHS will ask for them.

Good standing in Arizona is simpler than most states. Arizona LLCs do NOT file an annual report — there's no yearly state filing to keep an LLC active. (Only corporations file an annual report — \$45 for-profit. If you register a trade name / DBA, that's at the Secretary of State, azsos.gov, and renews every 5 years.)

3.2 Get your EIN (federal tax ID)

An EIN is your business's federal tax number — free from the IRS, and you need it for the bank, payroll, and AHCCCS.

1. **Go to IRS.gov** and search “Apply for an EIN online” (the EIN Assistant). It's free — never pay a third-party site.
2. **Apply online** with the responsible party's name and SSN/ITIN and your ACC entity info.
3. **Save the confirmation (CP-575)** — you get the EIN immediately.

3.3 Open a business bank account

What to have ready:

- EIN confirmation from the IRS
- Your ACC Articles of Organization and entity number
- Operating Agreement (LLC)
- A Beneficial Ownership form (owners of 25%+ and a control person)
- Government photo ID, SSN, DOB, home address for each owner/signer
- An opening deposit (often ~\$100)

Where to go: a major bank with lots of Arizona branches — Chase, Bank of America, Wells Fargo, or a local option like Arizona Financial Credit Union, Desert Financial Credit Union, or a community bank. Compare monthly fees and how to waive them, transaction/cash-deposit limits, branch access, online bill pay, and whether they offer a business line of credit.

3.4 Get your insurance — what you need, who sells it, and what to ask

An assisted living home cares for vulnerable adults in a building, so you need several kinds of coverage — not one policy:

Coverage	What it protects against	Must-have?
General Liability (GL)	A visitor/family slip-and-fall or property damage on your premises	Yes — foundational
Professional Liability (malpractice / E&O)	Claims about the CARE — supervision, med errors, neglect	Yes — top healthcare coverage
Abuse & Molestation	Allegations of abuse by staff or others (GL)	Yes — high exposure

	excludes this!)	
Commercial Property	The building, contents, equipment; add Business Interruption	Yes if you own; often if you lease
Commercial Auto (incl. hired/non-owned)	Transporting residents; staff driving own cars for work	Yes if you drive residents
Workers' Compensation	Staff injuries (lifting/transferring is high-risk)	Required in AZ once you have employees (verify)
Cyber Liability	A breach of protected health information (HIPAA)	Strongly recommended

Real places to get it

Buy through an independent agent/broker who accesses specialty carriers. Carriers that write senior-care/residential-care risk include Philadelphia Insurance (PHLY), CNA, Markel, Berkshire Hathaway GUARD, Crum & Forster, Selective, and Nationwide. Specialty programs/brokers for this niche include Caitlin Morgan, Gallagher, AssuredPartners, Professional Insurance Group, and XInsurance. To find a local Arizona agent, use the “Trusted Choice” Find-an-Agent directory at [trustedchoice.com](https://www.trustedchoice.com) (filter healthcare/senior-care), or ask a licensed peer. Verify each carrier writes assisted living in Arizona before relying on a quote.

What to ask the insurance agent

“Do you write assisted living homes / brain-injury directed-care facilities — which carriers?”

“Is professional liability included — claims-made or occurrence — and what limits?”

“Is abuse & molestation included, at what limit, sub-limit or full?”

“Are GL and professional liability shared limits or separate?”

“What limits does an ALTCS health plan or my landlord/lender require?”

“Is the carrier admitted or surplus lines in Arizona? What deductible on each line?”

3.5 Secure & zone the home

- ☐ Call your city/county PLANNING & ZONING office: is a residential assisted living use for [n] residents allowed at this address, or do you need a use permit? Arizona group homes for people with disabilities generally have residential-use protection — ask them to confirm in writing.
- ☐ Confirm the building can meet the Article 8 physical-plant specs (Part 4) and be made accessible.
- ☐ If leasing, make sure the lease allows your use and resident count.

3.6 Map ownership + fingerprint clearance

Identify every owner and controlling person — you disclose them on the ADHS application, and they (plus every manager, caregiver, contractor, and volunteer) must obtain a valid Arizona Fingerprint Clearance Card (Part 5.4) before providing services.

3.7 Get your NPI (National Provider Identifier)

An NPI is a free national ID payers use to identify you. Your FACILITY needs a Type 2 (Organizational) NPI:

- 1. Create an I&A account** at nppes.cms.hhs.gov (Identity & Access) for the person who'll manage the number (usually an owner).
- 2. Log into NPPES** and start a new application; choose Type 2 (Organizational).
- 3. Enter your legal name, EIN, and address** and pick the taxonomy that fits an assisted living / residential facility — confirm the exact taxonomy AHCCCS wants.
- 4. Submit — it's free** and usually processes in about 10 business days. Save your NPI; you'll need it for AHCCCS (Part 8).

PART 4

The Home — Physical Plant, Safety & Environment (Article 8)

In short: *Unlike some states, Arizona gives you real numbers. Here are the actual Article 8 requirements, what each one means in practice, and how the inspector checks it — build and set up to these, and confirm at your licensing inspection.*

“Physical plant” just means the building itself and everything fixed in it — the rooms, plumbing, heating/cooling, fire systems, and safety features. ADHS inspects all of it before it issues your license and again at renewal/complaint surveys. The rules live in the Arizona Administrative Code (A.A.C. Title 9, Chapter 10, Article 8). Below, each requirement is followed by what it means for you.

4.1 Bedrooms & space (R9-10-820)

- ☐ Private bedroom: at least 80 sq ft of floor space (not counting closet/bathroom)
- ☐ Shared bedroom: at least 60 sq ft PER resident; no more than two residents per bedroom
- ☐ Residential (apartment) unit: at least 220 sq ft for one occupant, +100 sq ft per additional occupant
- ☐ Each resident has their own bed (no bunk beds) and lockable or private personal storage
- ☐ A common area with space/furniture for recreation & socialization; a dining area with enough tables/chairs for everyone to eat at once
- ☐ An outdoor activity area with a hard surface for wheelchairs and available shade

What this means for you: a standard 3- or 4-bedroom Arizona house can usually work for a small directed-care home — but measure every bedroom before you sign a lease. A 10-by-8 bedroom is exactly 80 sq ft, so anything smaller can't be a private room. If you plan to share a room, you need 120 sq ft for two residents and you may never put a third bed in it. Brain-injury residents often use wheelchairs or walkers, so leave real turning room — the square-foot minimum is a floor, not a target.

4.2 Bathrooms (R9-10-820)

- **Toilets & sinks:** at least one working flush toilet (with seat) and one sink with running water for every 8 residents.
- **Bathing:** at least one working bathtub or shower for every 8 residents.
- **Safety:** grab bars at toilets and in tubs/showers, non-slip surfaces, and safety features as needed for residents using auxiliary aids (wheelchairs, walkers, transfer benches).

What this means for you: a typical two-bathroom house already meets the 1-per-8 ratio for a small home — the work is making the bathrooms SAFE. Budget for grab bars, a roll-in or transfer-bench shower, and a raised toilet seat. For brain-injury residents with impulsivity or poor safety awareness, add anti-scald valves and consider door hardware that a resident can't lock themselves behind in a crisis.

4.3 Environment, kitchen & storage (R9-10-819)

- **Temperature:** keep indoor temperature between 70°F and 84°F at all times (unless a resident controls their own room). Arizona summers make cooling non-negotiable — have a working, maintained A/C system and a backup plan for outages.
- **Hot water:** keep hot water at resident-use fixtures between 95°F and 120°F — hot enough to be useful, capped to prevent scalds. Set the water heater and verify with a thermometer; document it.
- **Kitchen & food:** a kitchen able to prepare meals that meet residents' dietary needs; store, prepare, and serve food safely; refrigeration at/below 41°F; a 3-day supply of non-perishable food and water for emergencies.
- **Hazardous materials:** store poisons, chemicals, cleaning supplies, and sharps separately from food and where directed-care residents cannot reach them — locked if residents have cognitive impairment.

- **Medications:** store medications in a locked area (a locked cabinet or room); refrigerated meds in a separate locked box in the fridge; controlled substances double-locked and counted.

What this means for you: think like an inspector walking your kitchen and utility closet. Chemicals under the sink, a fridge that runs at 45°F, or an unlocked med cabinet are the most common early citations. Fix these before you apply — put a thermometer in the fridge, a lock on the med cabinet, and every chemical up high or locked.

4.4 Brain-injury environmental design (why it matters here)

Beyond the code minimums, a brain-injury home should be set up for the residents' specific risks: clear, low-clutter walking paths to reduce falls; consistent, labeled locations for personal items (memory and orientation support); secured exits or door alarms if any resident wanders; reduced noise/visual overstimulation in common areas; and a quiet space a dysregulated resident can go to. None of this is optional in spirit — R9-10-819 requires the home be maintained to protect residents' health and safety, and directed care assumes you can keep an impulsive or disoriented resident safe.

4.5 Fire & disaster safety (R9-10-818)

- ☐ Smoke detectors in each bedroom, adjoining hallways, storage/laundry rooms, attached garage, and by the kitchen — battery or hard-wired with battery backup
- ☐ Test smoke detectors at least once a month and keep the documentation 12 months
- ☐ A fire extinguisher on each floor, inspected/serviced annually, and clearly marked exits
- ☐ A written disaster & evacuation plan covering resident relocation, medical-record and medication availability, and food/water
- ☐ Run a disaster drill on each shift at least once every 3 months, an evacuation drill at least once every 6 months, and review the plan at least every 12 months
- ☐ Keep every fire, building, and health inspection report in one dated compliance binder

Get the fire marshal in early. Your local fire department inspects for your occupancy type before ADHS licenses you. Brain-injury residents may not be able to self-evacuate, which affects the requirements — call the fire marshal's office while you're still choosing or modifying the home, not after. Contact info is in Part 11.

PART 5

Staff, the Manager & Required Training — by Position

In short: Each role has its own age, training, certification, and screening rules. Here they are, one by one — and the manager certification is the long pole, so start it early.

5.1 Certify your manager (NCIA Board — A.R.S. § 36-446.04)

Every assisted living facility must be run by a certified manager (who may be you). Here's exactly how to get certified:

1. **Be 21+ and hold a Fingerprint Clearance Card** (5.4).
2. **Complete a Board-approved manager training program** of at least 40 hours (covers resident-services, personnel, medication, legal management, and financial/physical-environment management), plus the caregiver program and adult CPR/first aid.
3. **Get the work experience** — 2,080 hours of paid work in a health-related field within the last 5 years.
4. **Pass the Board exam** (passing score is 75% on the Arizona exam).
5. **Apply & pay the fees.** On the May-2024 schedule: application \$183, exam \$183, certificate issuance \$183 (or prorated), renewal \$183 — verify current.
6. **Need to open sooner?** A temporary certificate (§ 36-446.06) gives a single, non-renewable 150-day window if you're qualified and haven't previously failed the exam — but it's automatically revoked if you fail.
7. **Renew biennially** with 24 continuing-education hours per 2-year cycle.

Do this first. You cannot operate or get licensed without a certified manager, and the 40-hour training + experience + exam take real time. Start the manager certification before anything else. NCIA Board: 602-364-2374, nciaboard.az.gov.

5.2 Caregivers

- **Age:** at least 18.
- **Required training (A.A.C. R4-33-703):** complete a Board-approved caregiver training program of at least 62 hours (50 hours of instruction + 12 hours of supervised skills), covering medication management (16 hrs), basic caregiver skills, infection control, care of the cognitively impaired, service plans, nutrition, fire/safety, and more; pass the exam at 80%. The caregiver certificate does not expire.
- **First aid & CPR:** current adult first aid AND CPR certification before providing services (Red Cross, AHA, or National Safety Council).
- **Screening:** a valid Fingerprint Clearance Card and an APS Registry check before client contact; TB clearance.

5.3 Assistant caregivers & coverage

- **Assistant caregivers:** at least 16; interact with residents only under the supervision of a manager or caregiver.
- **Coverage (assisted living HOME):** a manager or caregiver is present whenever a resident is present, awake except during nighttime hours; if not awake at night, staff must be able to hear and respond to a resident needing help. Keep a documented backup-staffing plan.
- **Brain-injury competence:** train all staff on cognitive stimulation, personal- and wander-safety, responding to sudden or out-of-control behavior, and communication strategies.

5.4 The Fingerprint Clearance Card (A.R.S. § 36-411)

Every owner, manager, caregiver, assistant caregiver, contractor, and volunteer with resident contact must obtain a valid Arizona Fingerprint Clearance Card, and apply within 20 working days of starting. Here's how:

1. **Apply through Arizona DPS** at the Public Services Portal, psp.azdps.gov — create an account and select the Fingerprint Clearance Card (care settings generally need the Level One card).

2. **Get fingerprinted** and pay the fee — \$67 (standard/Level One; \$65 for qualified volunteers).
3. **Also check the APS Registry** for every prospective and current employee (required since Jan 1, 2025), through the state's Centralized Background Check portal (cbc.az.gov) / AZ Care Check.
4. **If someone is awaiting a good-cause exception**, they may work only under the direct visual supervision of a card-holder.

***What this means for you.** Fingerprint cards and APS checks take time you don't control, so start them the moment someone accepts a job — not on their first shift. A card can be delayed weeks if there's a records match to resolve, and you may not let a new caregiver work unsupervised until the card is in hand. Build a 20-working-day buffer into every hire, keep a copy of each card and each APS/registry result in the personnel file, and diary the card's expiration so it never lapses.*

5.5 Before-hire screening checklist (every staff member)

- ☐ Valid Arizona Fingerprint Clearance Card in hand (or good-cause exception with direct visual supervision documented)
- ☐ APS Registry check cleared through cbc.az.gov / AZ Care Check (required since Jan 1, 2025)
- ☐ TB screening / clearance on file before resident contact
- ☐ Current adult first aid AND CPR certification (caregivers) before providing services
- ☐ Caregiver training completion (62 hr) or manager certification, as the role requires
- ☐ Signed job description, resident-rights orientation, and confidentiality/HIPAA acknowledgment
- ☐ Documentation of brain-injury competency training (cognitive stimulation, wander-safety, de-escalation)

5.6 Ongoing training & records to keep

Certification is not one-and-done. Keep first aid and CPR current, run periodic competency check-offs, and document every in-service. For a directed-care brain-injury home, plan recurring training on de-escalating sudden behavior, safe transfers and fall prevention, medication administration, elopement response, and reporting abuse or neglect. Your personnel records — cards, training certificates, TB, evaluations — are among the first things an ADHS surveyor pulls, so keep one organized file per employee and a simple roster showing every credential and its expiration date at a glance.

PART 6

Get Your ADHS Assisted Living License — Step by Step

In short: Apply in the ADHS LMS portal with your manager, policies, floor plans, and fingerprint clearances; pass the inspection; the license is perpetual (you pay an annual fee to keep it).

6.1 Start the application in the LMS

ADHS takes applications only electronically through the Licensing Management System (LMS) at facility-licensing.azdhs.gov — paper forms are no longer accepted. Create your account and start the Assisted Living Home application.

What the application & checklist require (have these handy):

- Ownership and controlling-person information
- Your certified manager's designation and credentials
- Floor plan(s) showing the square-footage and bathroom-ratio compliance (Part 4)
- Fire-safety documentation (extinguisher maintenance, smoke-detector placement/testing) and disaster/evacuation drill records
- Fingerprint Clearance Cards and APS-registry verification for staff; TB screening
- Personnel records + caregiver-training completion
- Your written policies & procedures (the 23 required-policy areas of R9-10-803(C))
- Scope of services + level(s) of care (Directed Care), residency agreements, and resident service plans
- Local zoning approval obtained BEFORE applying

6.2 Attachments checklist

- ☐ Ownership / controlling-person disclosures
- ☐ Certified manager designation & certificate
- ☐ Floor plans demonstrating R9-10-820 compliance
- ☐ Fire-safety + disaster/evacuation documentation
- ☐ Fingerprint Clearance Cards + APS registry checks + TB screening
- ☐ Policies & procedures (23 areas of R9-10-803(C)) + referenced forms
- ☐ Scope of services / level of care; residency agreements; service plans
- ☐ Zoning approval

Where your manual & forms fit. The application requires complete Article 8 policies and procedures. PolicyReady's Assisted Living / Brain-Injury Policy & Procedure Manual and Forms Packet cover the R9-10-803(C) policy areas and are built to the accreditation frameworks — customize them before you apply.

6.3 Pay the fees & pass the inspection

- **Application fee:** \$50, non-refundable, with the initial application (R9-10-106(B)).
- **Annual license fee:** \$280 base + \$70 per licensed bed. So a 6-bed home ≈ \$700/year; a 10-bed home ≈ \$980/year (R9-10-106).
- **The license is PERPETUAL** — it doesn't expire on a fixed term. You keep it by paying the annual fee tied to your anniversary date; there's a 30-day grace period, then a \$250 late fee, and it voids if still unpaid (R9-10-107).
- **Pre-licensure inspection:** an ADHS surveyor visits after the administrative review — expect a policies review, personnel-record review, and an environmental tour that includes measuring bedrooms. Clear any deficiencies and ADHS issues the license (often 2–5 business days after a no-deficiency survey). Plan roughly 120 days end to end.

PART 7

The Resident's Journey — Referral to Discharge

In short: An ALTCS member is referred by their health plan; the case manager sets the level of care and the service plan; you deliver directed care, review it, and plan for the least-restrictive setting. Here is the whole journey, stage by stage — what you do, what to have ready, and who's involved.

Stage 1 — Referral comes in

A referral comes from an ALTCS health plan / case manager, or a hospital or Barrow discharge planner (Part 10). Someone calls or emails asking whether you have a bed for a specific person.

What you do:

- Take the basic information (age, diagnosis, level of care, funding source, target move date). Ask for a copy of the person's current assessment and service plan. Tell them your license level (Directed Care) and current openings.

Stage 2 — Pre-admission screening (can you safely serve this person?)

Before you say yes, confirm the resident actually fits your license, your capacity, and your abilities. A directed-care home cannot accept residents whose needs exceed directed care — for example, someone who is bed- or chair-confined and can't be moved, or who has a stage 3–4 pressure sore — except under narrow, documented medical conditions with a plan.

What to review:

- The person's diagnoses and behaviors (aggression, elopement/wandering, swallowing/choking risk); their mobility and transfer needs; their medications, including any that require special monitoring; and whether your trained staffing can meet all of it safely. When in doubt, ask the case manager and the resident's physician.

Stage 3 — The ALTCS case manager sets the level of care

For ALTCS members, the ALTCS case manager — not you — determines the level of care using the HCBS Needs Tool (HNT) and documents it in the member's Person-Centered Service Plan (PCSP). The PCSP authorizes the directed-care assisted-living service and any add-ons (physical/occupational/speech therapy, transport, home health, behavioral health — each billed separately). If acuity later changes, the case manager updates the level, which can change your rate.

Stage 4 — Admission & the residency agreement

On move-in, the resident (or their designated representative) signs a residency agreement (required by R9-10-807) covering the services you'll provide, resident rights, fees, who pays room and board, and how residency can end. Give a rights orientation and open a resident record.

The resident record must include:

- Health information and diagnoses; a current medication list with allergies; advance directives / health-care power of attorney; the physician's and case manager's contact info; emergency contacts and the designated representative; and the signed agreement and consents.

Stage 5 — Build the service plan (the directed-care roadmap)

Within your policy timeframe, develop and then follow the resident's service plan — the day-to-day plan for THIS person. For a brain-injury resident it typically covers cognitive-stimulation activities to maximize functioning, personal-safety and wander-safety measures, help with all activities of daily living (bathing, dressing, toileting, transfers, eating), medication assistance or administration, nutrition and any special diet, and behavior supports. Coordinate it with the resident's physician and the ALTCS case manager, and have the resident/representative sign off.

Stage 6 — Daily life & directed care

Deliver the plan: structured daily routines, ADL support, cognitive and social activities, community outings as appropriate, and safe medication practices — all mapped to the resident's goals. Maintain awake or hear-and-

respond coverage per your staffing rules, and document nighttime checks on directed-care residents. Keep progress notes so you can show the plan is being followed.

Stage 7 — Reviews, incidents & rights

Review and update the service plan as needs change and whenever the case manager reassesses. Run your quality-management program with incident reports (R9-10-803(C)); protect resident rights (dignity, privacy, the right to complain, health-care directives); and report any suspected abuse, neglect, or exploitation to Adult Protective Services immediately. Every incident and every rights complaint gets documented and followed up.

Stage 8 — Discharge & transition

When a resident's needs change or their goals are met, coordinate a safe transition with the case manager and family — to a less-restrictive setting, a higher level of care, or back to the community — following your termination-of-residency policy and giving proper written notice. Send the receiving provider the records they need for a warm handoff, and document the whole transition.

***What this means for you.** A safe, well-documented discharge protects both the resident and your license. Arizona's rules limit when and how you may end a residency, so never discharge abruptly — involve the ALTCS case manager early, give the written notice your residency agreement and R9-10-807 require, and keep the resident safe until the receiving setting is ready. A brain-injury resident may not understand or agree with the move, so plan extra orientation and involve the designated representative. Rushed or undocumented discharges are a common source of complaints and citations.*

Discharge & transition records checklist

- ☐ Written termination-of-residency notice, dated, with the reason and required notice period
- ☐ Case manager and designated representative notified and involved in the plan
- ☐ Current medication list, allergies, and any physician orders sent to the receiving provider
- ☐ Copy of the service plan, recent progress notes, and advance directives / health-care POA
- ☐ Resident's personal belongings and any personal-needs funds reconciled and returned
- ☐ Transportation arranged and confirmed for the move date; receiving setting ready
- ☐ Discharge summary and the whole transition documented in the resident record

A note on the whole journey

Every stage above turns into paper you must be able to produce: the referral and pre-admission screening, the signed residency agreement, the person-centered service plan and its reviews, daily progress and nighttime-check notes, incident reports, and the discharge summary. Run the journey as one continuous record for each resident. If you can hand a surveyor a single, complete file that tells the resident's story from referral to discharge, you have met both the ADHS documentation standards and what your ALTCS plan expects — and you have the evidence that your directed care is actually being delivered.

PART 8

How You Get Paid — ALTCS, Step by Step

In short: Enroll with AHCCCS, contract with the ALTCS health plans, and understand the capped rate. ALTCS pays for care — never room & board. Verify each resident's coverage before admission.

Summary of the sources:

Source	What it is	Key fact
ALTCS (via a health plan)	Arizona Medicaid long-term care, managed care	Pays the directed-care service; you bill the member's plan under contract
Private pay	Family/self-pay	Always covers room & board; may cover all care for non-ALTCS residents
Veterans (VA)	Aid & Attendance, VA community residential care	May help fund care — verify eligibility

8.1 Enroll as an AHCCCS provider (APEP)

1. **Get your NPI first** (Type 2 — Part 3.7).
2. **Enroll in APEP** (the AHCCCS Provider Enrollment Portal) at azahcccs.gov/APEP. Provide your NPI, a certified IRS W-9 (your EIN), your current ADHS license, and ownership/controlling-interest disclosures.
3. **Pick your provider type.** An assisted living HOME enrolls as Provider Type 36 (Assisted Living Home); a center is Type 49 — confirm on the current AHCCCS provider-type list.
4. **Complete screening.** AHCCCS assigns a risk level (limited / moderate / high); facility types often get a site visit. Enrollment help: 1-800-794-6862.
5. **Revalidate every 4 years.**

AHCCCS/the plan will ask you:

- ADHS license type & bed capacity and which level(s) of care are on your license; NPI + W-9; ownership disclosures; fingerprint/background + APS/registry checks; staffing model & DCW training; admission/discharge criteria; HCBS Settings Rule compliance.

You should ask:

- Which provider type covers an assisted living home? What risk category am I (site visit? fee?)? Do I need to be AHCCCS-enrolled before a plan will contract me (yes)?

8.2 Contract with the ALTCS health plans

Enrollment lets you exist in the system; the plan CONTRACT is what pays you. The current ALTCS Elderly & Physical Disability plans (contracts effective Oct 1, 2025) are:

- **Arizona Complete Health–Complete Care Plan (LTC):** AzCHNetDevALTCS@azcompletehealth.com · azcompletehealth.com
 - **Banner–University Family Care:** “Join Our Network” form at bannerhealth.com/bhpprovider/network
 - **Mercy Care:** Letter of Interest + W-9 to MercyCareNetworkManagement@MercyCareAZ.org · mercycaresaz.org
 - **UnitedHealthcare Community Plan:** Onboard Pro at onboard-pro.uhcprovider.com
1. Complete AHCCCS enrollment first (get your AHCCCS provider ID).
 2. Submit each plan's interest form / Letter of Intent with your W-9, NPI, AHCCCS ID, service area, services, and level(s) of care.
 3. Complete credentialing (often via CAQH).
 4. Negotiate and sign the provider agreement and rate; get an effective date before you serve/bill members.

8.3 The rate & what's covered

Assisted-living services bill under HCPCS code T2031 (“Assisted living; waiver, per diem”), which AHCCCS treats as a benchmark/negotiated rate rather than a single posted per-diem — so your actual rate is set in each plan contract. AHCCCS publishes a Capped Fee Schedule (updated about October 1 each year) that sets the ceiling; a negotiated rate generally can’t exceed it.

Room & board is NOT covered by ALTCS. ALTCS pays only the care/service component (AMPM 1230-A). The resident pays room and board from their own income (a “Share of Cost”), up to program limits. Build that into your admission and your resident agreement.

How the money actually works — a worked example

Two separate streams pay for one resident, and you have to keep them straight:

- **Stream 1 — the care, paid by the ALTCS plan:** the plan pays your contracted per-diem for the directed-care service under T2031. Say your contract is a per-diem — you bill the plan for each day the resident is in your care.
- **Stream 2 — room & board, paid by the resident:** the resident pays room and board from their own income (Social Security, pension). ALTCS limits how much of their income they keep for personal needs; the rest goes to their care as a “Share of Cost.” You collect room & board directly from the resident or their representative.

So a single bed can bring in, for example, the plan's daily care rate PLUS a fixed monthly room-and-board amount from the resident — but never room & board from ALTCS. Get the exact per-diem and the room-and-board figure in writing before you admit, and confirm the resident's Share of Cost with the case manager so there's no surprise.

How you actually bill (the claims cycle)

1. **Verify eligibility** before and during the stay — check the member's ALTCS enrollment and plan on the AHCCCS portal each month; enrollment can change.
2. **Get the authorization** — the PCSP/plan authorization is what lets you bill; confirm the units and dates approved.
3. **Capture EVV where required** — Electronic Visit Verification may apply to certain services; ask the plan whether your service needs EVV and which system to use.
4. **Submit the claim** — electronically (an 837 professional claim) through the plan's portal or clearinghouse, using your NPI, the T2031 code, dates, and units.
5. **Watch timely filing** — AHCCCS's baseline is 6 months from the date of service, but a plan contract can be shorter; bill promptly and reconcile remittances.

Ask each plan:

- The daily/monthly rate for a directed-care assisted living home and whether it's tied to the capped schedule or negotiated; how the case manager sets the level and what triggers a rate change; the authorization and claims process (portal, 837, EVV); the timely-filing deadline (AHCCCS baseline is 6 months — confirm any shorter contract deadline); how the member's room-and-board / Share of Cost is handled; and any enhanced-rate tier for high-acuity brain-injury residents.

8.4 Private pay & veterans

- **Private pay:** non-ALTCS residents (and every resident's room & board) pay privately — set a clear rate and a written residency/financial agreement; have counsel review it.
- **A note on DDD:** Arizona's Division of Developmental Disabilities generally does NOT cover adult-onset traumatic brain injury (DDD requires a qualifying condition with onset before age 18) — brain-injury adults access long-term care through ALTCS, not DDD.
- **Veterans:** VA Aid & Attendance and VA community residential care may help fund a veteran's care — verify current eligibility with the VA.

PART 9

What It Costs & How Long It Takes

In short: Budget the home, the manager certification, caregiver training, fingerprint cards, and the ADHS fees — and consider accreditation. Plan ~120 days through licensing, then contracting.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
The home	Lease deposit + first month (or purchase); accessibility mods; furnishings; life-safety systems	Varies by market/size
Manager certification	NCIA Board 40-hr training + exam + fees (\$183 each, verify)	~Several hundred \$
Caregiver training	62-hr program per caregiver + first aid/CPR	Per person
Fingerprint Clearance Cards	\$67 each (owner, manager, every caregiver)	\$67 × people
Insurance	GL + professional + abuse + property + auto + workers' comp (annual)	Get quotes
ADHS fees	\$50 application + \$280 + \$70/bed annual	10-bed ≈ \$980/yr
Business setup	ACC filing ~\$85, EIN (free), NPI (free), bank, legal	Low hundreds + legal
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional but recommended)	CARF or Joint Commission (see 9.2)	Low-to-mid five figures over the cycle

9.2 If you pursue accreditation (recommended for brain injury)

- **CARF:** roughly a \$995 application fee plus about \$1,525 per surveyor, per day (a small program's survey is ~2–3 days), on a 3-year cycle with an annual conformance report. CARF offers a Brain Injury specialty. (888) 281-6531, carf.org.
- **The Joint Commission:** an annual fee plus a survey fee, priced from your services and census, on a 3-year cycle. Request a quote: 630-792-5115, jointcommission.org.
- **It's staged:** both require you to be operating and serving residents before survey — CARF wants about six months first — then an initial on-site survey, then a decision (CARF terms run 3-year, 1-year, or provisional). Plan ~12–18 months if you pursue it.

9.3 Timeline

Early	Manager certified	Start the 40-hr training + experience + exam right away
Weeks	Business & home ready	Entity, insurance, home to Article 8 specs, zoning confirmed
Weeks	Staff & policies ready	Caregivers trained (62 hr), FCCs, P&P adopted
~120 days	ADHS application → license	LMS application + attachments → inspection → license
Then	Enroll & contract	AHCCCS APEP + ALTCS plan contracts
Ongoing	Referrals & full census	ALTCS case managers + discharge planners

PART 10

Getting Your First Residents

In short: ALTCS members reach you through their health plans and case managers, and through hospital and Barrow discharge planners. Build those relationships before you open.

10.1 The referral sources

- **ALTCS health plans & case managers:** once contracted, the plans and their case managers refer members who need directed care and are at the nursing-facility level of care. Tell each plan's placement/case-management team your openings, level of care, and brain-injury capability.
- **Hospital & rehab discharge planners:** the biggest pipeline for brain injury — the trauma centers and neuro-rehab hospitals below discharge patients who need a directed-care home. Introduce yourself, leave a one-pager, and ask how to get on their placement list.
- **Brain Injury Alliance of Arizona (BIAAZ):** a resource-facilitation and referral partner — Helpline 1-888-500-9165, main 602-508-8024, biaaz.org.

10.2 Trauma & neuro-rehab centers (verify before relying)

Metro	Program (referral source)	Contact (verify)
Phoenix	Dignity Health St. Joseph's / Barrow Neurological Institute	602-406-3000; Barrow Neuro-Rehab 602-406-1600
Phoenix	Banner – University Medical Center Phoenix	602-839-2000
Phoenix	Valleywise Health Medical Center (Level 1 + burn)	602-344-5011
Scottsdale	HonorHealth Osborn + HonorHealth Rehabilitation Hospital	480-882-4000; rehab 480-800-3900
Scottsdale/Mesa	Encompass Health Rehabilitation (Scottsdale / East Valley)	480-551-5400 / 480-567-0350
Tucson	Banner – University Medical Center Tucson	520-694-0111
Tucson	Encompass Health Rehabilitation Institute of Tucson	520-325-1300

Ask each referral source:

- How do you decide where to place brain-injury patients who need a directed-care home? What do you need from me to be on your placement list (license level, capability, availability)? Who is the discharge planner / case manager I should stay in touch with?

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the manager & the building

Who	Contact	What to verify / ask
ADHS Residential Facilities Licensing	602-542-1025 · facility-licensing.azdhs.gov	Current license fees; the application & attachments; the inspection; the LMS process
NCIA Board (manager cert)	602-364-2374 · nciaboard.az.gov	Approved manager training; the exam; current fees; temporary certificate; CE
AZ DPS — Fingerprint Clearance Card	602-223-2279 · psp.azdps.gov	Which card level; cost (\$67); the 20-day rule; good-cause exception
DES Adult Protective Services (APS Registry)	cbc.az.gov / AZ Care Check	How to run the required APS Registry checks on staff
City/County Planning & Zoning	your jurisdiction	Allowed use at your address; group-home protection — in writing
Local Fire / Building Dept.	your jurisdiction	Fire-safety requirements & inspection for your occupancy

Business, insurance & billing IDs

Who	Contact	What to verify / ask
AZ Corporation Commission	602-542-3026 · ecorp.azcc.gov	Forming your LLC (\$85); publication requirement in your county
AZ Secretary of State	602-542-6187 · azsos.gov/business	Trade name / DBA if you use one
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	nppe.cms.hhs.gov	Your Type 2 NPI and the right taxonomy
Insurance agent (Trusted Choice)	trustedchoice.com ; carriers in Part 3.4	Coverages & limits; abuse & molestation; plan-required limits
Business bank	Chase/BofA/Wells; Desert Financial, Arizona Financial CU	Account fees, limits, business line of credit

Funding & referrals

Who	Contact	What to verify / ask
AHCCCS Provider Enrollment (APEP)	azahcccs.gov/APEP · 1-800-794-6862	Provider type (36) & risk level; enrollment steps
ALTCS health plans	AZ Complete Health, Banner-UFC, Mercy Care, UnitedHealthcare (Part 8.2)	Contracting; the directed-care rate vs. the capped schedule; auth & claims; timely filing; room-and-board handling
AHCCCS Capped Fee Schedule	azahcccs.gov (HCBS rates) · FFSRates@azahcccs.gov	The current T2031 ceiling & any directed-care add-ons
Accreditor — CARF / TJC	CARF (888) 281-6531 · TJC 630-792-5115	Cost quote; the ~6-month operating requirement; Brain Injury specialty

Advocacy, oversight & referral partners

Who	Contact	Role
AZ Long-Term Care Ombudsman	602-542-6454, ext. 9 · des.az.gov/LTCOP	Independent resident-rights advocate — post for residents
DES Adult Protective Services	report abuse/neglect via DES APS	Mandatory reporting; the registry
Brain Injury Alliance of Arizona	1-888-500-9165 · 602-508-8024 · biaaz.org	Referrals, resource facilitation, client-assistance fund
Trauma / neuro-rehab hospitals	see Part 10	Your primary discharge-planner referral sources

You can do this

Opening an Arizona brain-injury assisted living home is a real build — a licensed building to Article 8 specs, a certified manager, trained caregivers, and vulnerable residents — but it's a defined path, and Arizona's directed-care level is made for exactly these residents. Take it one rung at a time: certify your manager, form the business, prepare the home and team, get licensed by ADHS, enroll and contract with the ALTCS plans, and earn referrals from the plans and the trauma centers. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in A.A.C. Title 9, Ch. 10, Article 8, A.R.S. Title 36, and current AHCCCS/ALTCS information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with ADHS, the NCIA Board, AHCCCS/ALTCS, and qualified counsel before you rely on it. Confirm your payment rate in your provider contract with each ALTCS health plan and check the current AHCCCS Capped Fee Schedule.